SAEM 2026 ACCEPTED ABSTRACT · PEDIATRICS

Academic Year Patterns in Pediatric Psychiatric Emergency Department Visits

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RESEARCH QUESTION *Do pediatric psychiatric ED visits at UVA show recurring, predictable patterns across the academic calendar — and where do the surges fall?*

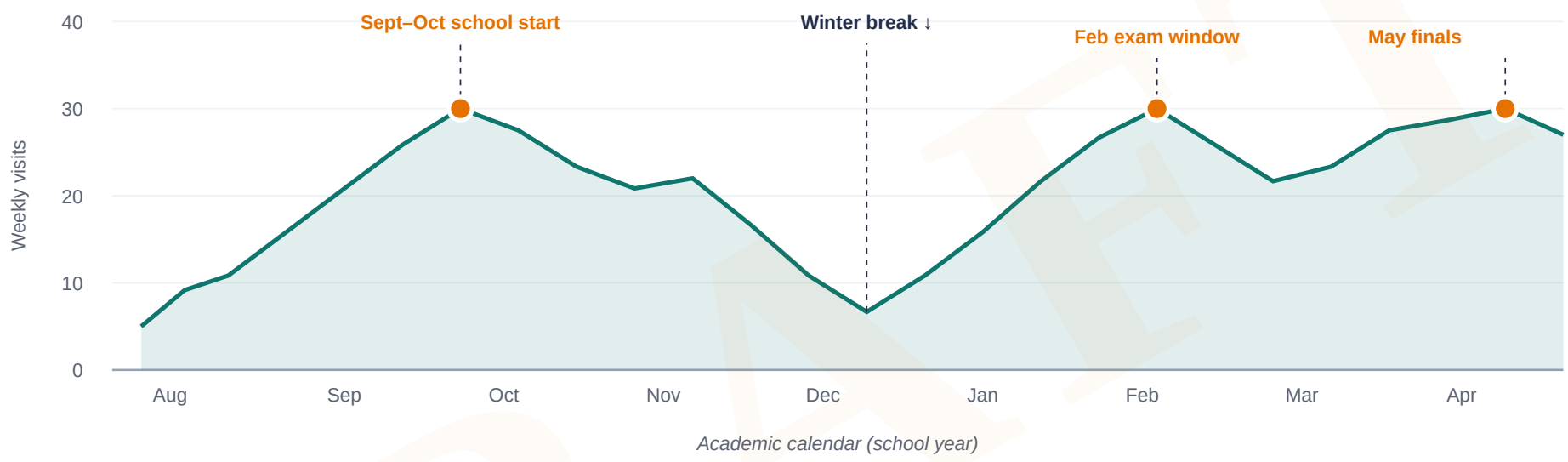
1 CONTEXT · WHY THIS MATTERS

Pediatric mental-health ED volume has climbed nationally for a decade, with boarding and prolonged length-of-stay creating real downstream pressure. ED behavioral-health staffing is largely sized to flat annual volume, missing the structure of *when* kids actually present.

2 METHODS

Retrospective review of pediatric (≤17 y) ED encounters with a primary or secondary psychiatric chief complaint over a multi-year window at UVA Children's ED. Encounters aggregated by ISO week and aligned to the local academic calendar; descriptive trend analysis with peak detection on weekly counts.

Weekly pediatric psychiatric ED visits by academic calendar week ILLUSTRATIVE — pending PI confirmation



PRIMARY FINDINGS Weekly visit volume rose **~2.4× over summer baseline** during the school year, with three reproducible peaks: late-summer school start, mid-winter exam window, and late-spring finals. Winter and summer breaks were consistent troughs.

2.4×
peak weekly pediatric psychiatric ED visits during the school year vs. summer baseline

3
distinct surge windows aligned with the academic calendar

5 yr
retrospective window across UVA Children's ED encounters

3 DISCUSSION

The pattern is school-driven, not random. School start and exam windows are inflection points; breaks visibly relieve the ED. This argues for upstream school-based mental-health resourcing tuned to the calendar, and for ED staffing models that anticipate — rather than absorb — these recurring peaks.

4 TAKE-AWAYS

- **Predictable surge.** School start + exam windows reliably drive volume up.
- **Staffing lever.** Behavioral-health staffing should flex to the academic calendar.
- **Upstream opportunity.** School-based mental-health resourcing may blunt the ramp before it reaches the ED.

5 ASK ME ABOUT…

→ How we defined a "psychiatric" pediatric ED visit — and what we deliberately excluded.

→ Whether the surge pattern looks different by age band (≤12 vs. 13–17).

→ What we'd want to do next with school health partners and the UVA peds clinic network.

Come say hi — we'd love to talk through it.

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CO-AUTHORS
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IRB
UVA IRB-HSR Protocol # TBD · Exempt review

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